

Public Health Emergency Operations Center
"COUSP DRC"

MVE-17 Incident Management System

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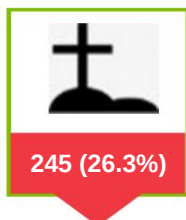
Situation Report of the 17th Ebola Virus Disease Outbreak / DRC

SitRep N°35/MVB_18/06/2026

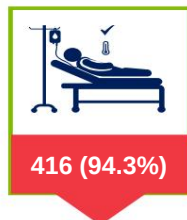
Provinces affected (3)	ITURI, NORTH KIVU & SOUTH KIVU
Affected Health Zones (34)	- Ituri (22/36): Aru, Aungba, Bambu, Bunia, Damas, Drodoro, Gety, Kilo, Komanda, Lita, Logo, Mambasa, Mangala, Mongbwalu, Nizi, Nyankunde, Rimba, Rwampara, Tchomia, Kambala, Nia-Nia and Fataki. - North Kivu (11/34): Beni, Butembo, Goma, Kalunguta, Katwa, Kyondo, Oicha, Masereka, Vuhovi, Mabalako and Musienene. - South Kivu (1/34): Miti-Murhesa
Reporting date	June 18, 2026
Publication date	June 19, 2026



Total confirmed cases
for the 3 provinces



Cumulative deaths
among confirmed cases
and case fatality rate



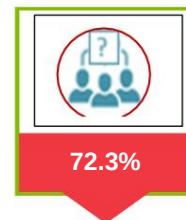
Patients in
isolation -
hospitalization



Cumulative number
of recoveries



Suspected cases of
the day



Contact tracing rates
for the 3 provinces

* Data updated following the DHIS2 database cleaning and harmonization process, including the consolidation of information from health zones, laboratories and structures case management.
continuous harmonization and consolidations.

Suspicious deaths occurring in the community are subject to investigation.
further investigation with a view to possible reclassification (Non-cases, probable cases)

1. HIGHLIGHTS

- **Thirty-six (36) new confirmed cases including 12 deaths**, were reported in the provinces of **Ituri (35 including 12 deaths) and North Kivu (1)** as of June 18, 2026. These confirmed cases are distributed in the health zones of **Mongbwalu (9), Nyankunde (9), Bunia (6), Rwampara (4), Mangala (2), Aungba (2), Lita (2), and Nizi (1)** in the province of Ituri as well as those of **Beni (1)** in the province of North Kivu.
- **Two (2) patients** with Ebola virus disease have been declared cured after receiving negative control tests.
- **Arrival of 3 national-level ministers to assess the response actions to Ebola virus disease.**

2. EPIDEMIOLOGICAL AND OPERATIONAL CONTEXT

Ituri province, located in the northeast of the Democratic Republic of Congo, shares a long and porous border with Uganda and South Sudan. For over two decades, it has faced a chronic humanitarian crisis linked to armed conflict and recurring population displacements. Its population is estimated at over 8 million, including more than one million internally displaced persons. The Mongbwalu health zone, considered the starting point of the epidemic, is located in the Djugu territory, 70 km from Bunia. This area is characterized by the presence of armed groups, frequent population movements towards Uganda, and numerous artisanal mining sites attracting migrant workers. In September 2018, Ituri province had already been affected by the 10th Ebola virus disease (EVD) outbreak, which was then raging in North Kivu. The current epidemic, caused by the Bundibugyo ebolavirus, was officially declared on May 15, 2026, by the Minister of Public Health. The three affected provinces have a population

total estimated at nearly 15 million inhabitants, with massive internal displacements and cross-border flows to neighboring countries.

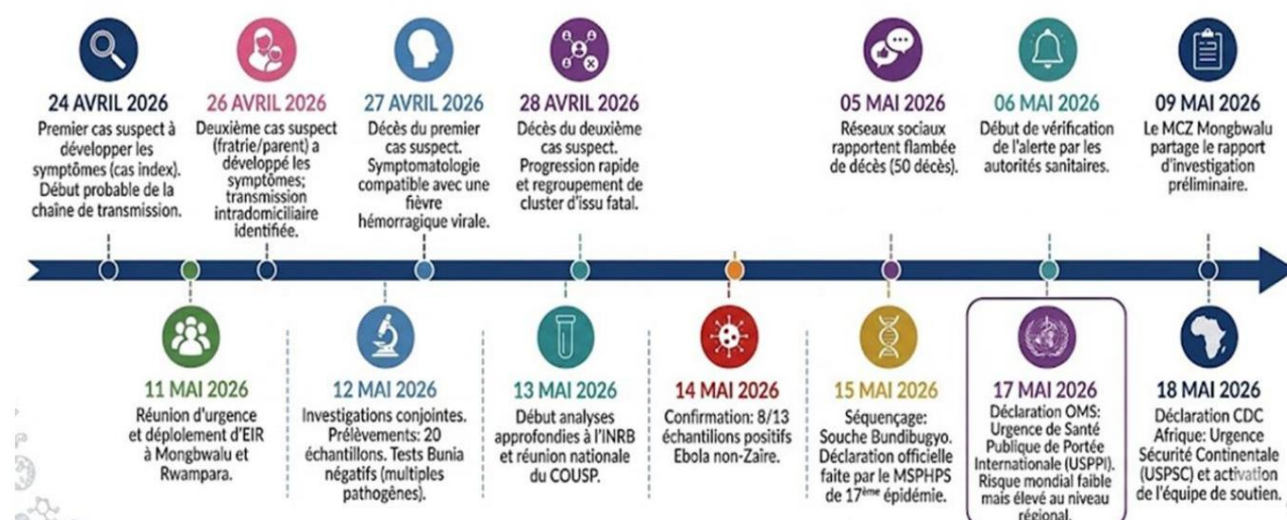


Figure 1. Chronology of events of the Ebola-Bundibugyo virus disease epidemic.

3. DETAILED EPIDEMIOLOGICAL ANALYSIS

3.1 Spatial distribution and active hotspots as of June 18, 2026

Table 1. Distribution of confirmed cases and deaths by affected province as of June 18, 2026.

Province	Confirmed cases	Deaths (confirmed)	Lethality	Affected health zones	New confirmed cases (24h)
Ituri	853	199	23.3%	22 out of 36 (61.1%)	35
North Kivu	77	45	58.4%	11 out of 34 (32.3%)	1
South Kivu			33.3%	1 out of 34 (2.9%)	0
Total	3,933	1 245	26.3%	34 out of 104 (32.7%)	36

As of June 18, 2026, no new health zones were affected. The number of affected health zones remains at 34/104 (section 3.1). Furthermore, the province of South Kivu has not reported any confirmed cases since May 26, 2026 (Table 1). There has been no recent transmission in South Kivu, but vigilance remains essential.

3.2 Distribution by health zone (Cumulative as of 18/06/2026)

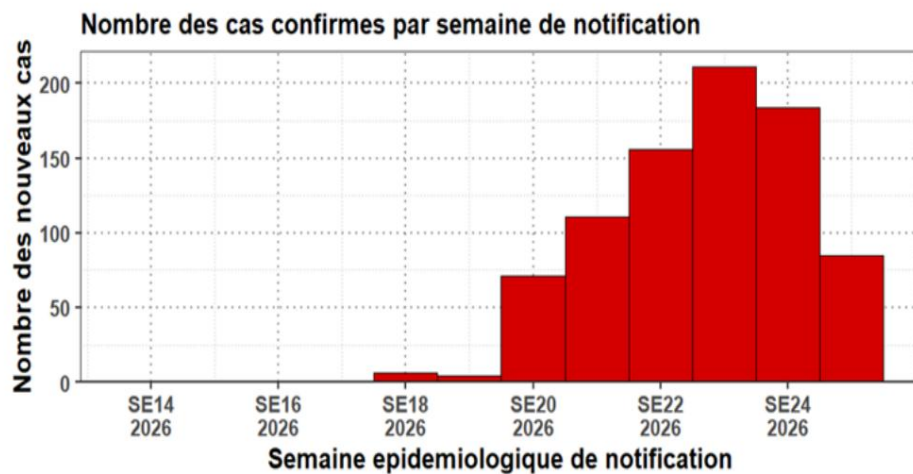
• **Ituri Province (853 cases):** Bunia (253), Rwampara (199), Mongbwalu (198), Nyankunde (77), Nizi (26), Lita (20), Komanda (10), Bambu (9), Kilo (7), Mangala (7), Tchomia (5), Damas (4), Aungba (6), Rimba (3), Aru (3), Mambasa (2), Logo (2), Drodoro (1), Gety (1), Kambala (1), Nia-Nia (1) and Fataki (1).

Seventeen (17) additional cases are awaiting precise breakdown by health zone in the province.

• **North Kivu Province (77 cases):** Butembo (25), Katwa (24), Beni (16), Oicha (3), Kalunguta (2), Kyondo (2), Goma (1), Masereka (1), Vuhovi (1), Mabalako (1) and Musienene (1).

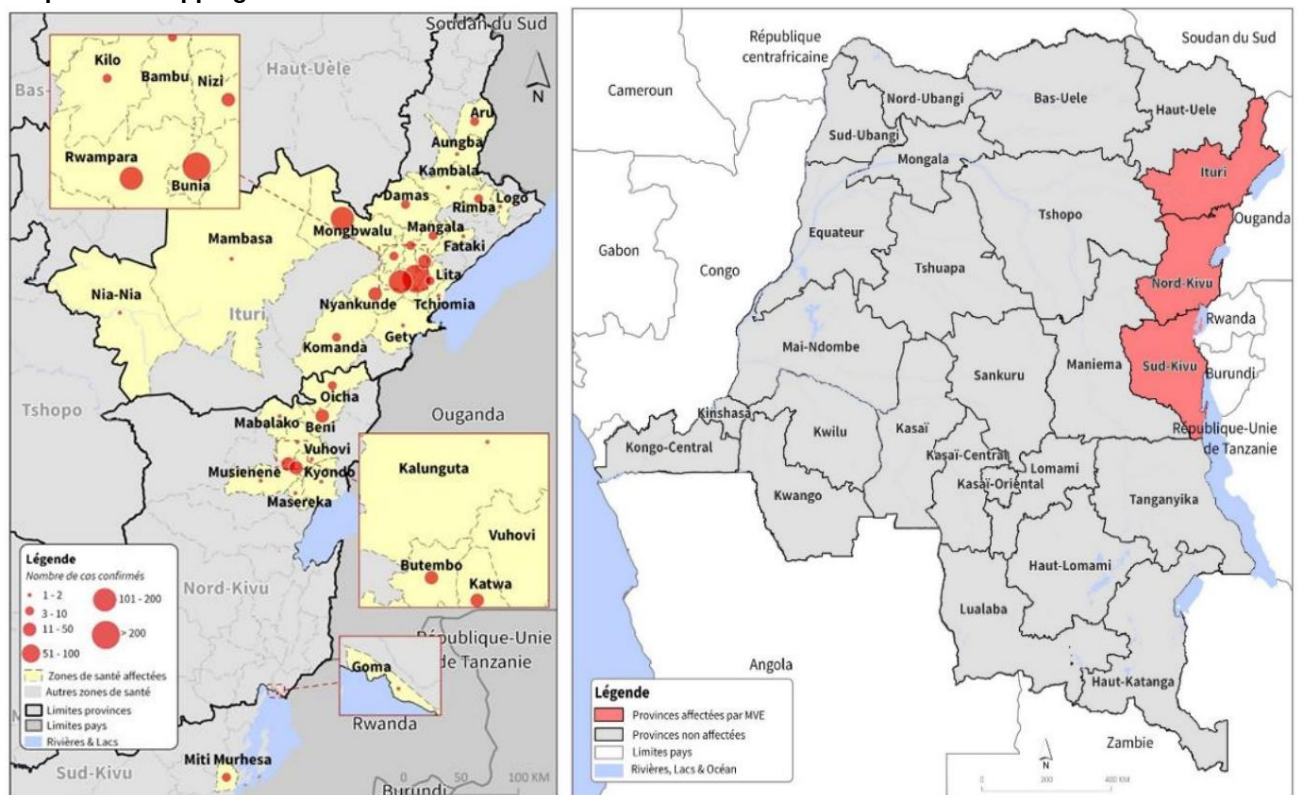
• **South Kivu Province (3 cases):** Miti-Murhesa (3).

3.3 Temporal Analysis: An increasing number of confirmed cases is observed from one week to the next, reflecting continuous transmission of the disease within the community. Rapid geographical expansion is also observed. The epidemic remains possible if public health actions are not implemented quickly.



Source : DHIS2 Tracker - Riposte MVE, juin 2026. Données incluses dans l'analyse : n cas confirmés = 828.

3.4 Epidemic mapping



Figures 2 & 3. Spatial distribution of confirmed Ebola cases by affected health zones and in the three affected provinces as of June 18, 2026

Table 2. Confirmed cases and deaths by province and health zone (June 18, 2026)

Province / Health Zone	Cumulative confirmed cases (n)	Cumulative confirmed deaths (n)	Case fatality rate (CFR, %)
ITURI	853	199	23.3%
Bunia	253	42	16.6%
Rwampara	199	35	17.6%
Mongbwalu	198	87	43.9%
Nyankunde	77	7	9.1%
Nizi	26	5	19.2%
Lita	20	6	30.0%
Komanda	10	3	30.0%
Bamboo	9	2	22.2%
Kilo	7		14.3%
Mangala	7	1	57.1%
Tchomia	5	4	0.0%
Damascus	4	0	0.0%
Aungba	6	0	33.3%
Rimba	3	2	0.0%
Aru	3		33.3%
Mambasa	2		50.0%
Logo	2	0	0.0%
Drodro			100.0%
Gety		1	0.0%
Kambala			100.0%
Nia-Nia	1 1 1 1	1 0 1 0 1 1	100.0%
Fataki	1	0	0.0%
Other areas not yet identified	17	0	0.0%
NORTH KIVU	77	45	58.4%
Butembo	25	11	44.0%
Katwa	24	16	66.7%
Blessed	16	12	75.0%
Oicha	3	2	66.7%
Kalunguta	2		50.0%
Kyondo	2		50.0%
Goma		1	0.0%
Masereka	1 1	1	0.0%
Vuhovi	1		100.0%
Mabalako	1	0	0.0%
Musienene	1		100.0%
SOUTH KIVU	3	0 1 0 1 1	33.3%
Miti-Murhesa	3	1	33.3%
TOTAL 26.3%	933	245	

As of June 19, 2026 (data reported as of June 18, 2026), the epidemiological situation of the 17th Ebola Virus Disease (EVD) outbreak in the Democratic Republic of Congo has reached a cumulative total of **933 confirmed cases** and **245 deaths**, expressing an overall case fatality rate of **26.3%**.

Provincial dynamics and case concentration

The province of **Ituri** remains the absolute epicenter of the epidemic, accounting for **91.4%** of cases (n=853) and **81.2%** Deaths (n=199) represent a case fatality rate of **23.3%**. The most affected health zones are Bunia (253 cases), Rwampara (199 cases), and Mongbwalu (198 cases), encompassing three-quarters of the reported cases (n=650) in the province. The Bunia and Rwampara health zones have moderate case fatality rates (16.6% and 17.6%, respectively), while the **Mongbwalu** zone shows marked severity with a rate of **43.9%** (87 deaths out of 198 cases), suggesting persistent challenges in early management or access to care. Furthermore, 17 additional cases have been reported in as-yet-unidentified health zones, without associated deaths, which could indicate a geographic expansion requiring further investigation.

Severity of the epidemic in North Kivu

North Kivu province presents the most concerning profile in terms of severity, with **77 confirmed cases** .

With **45 deaths**, the provincial case fatality rate reached **58.4%**, more than double the national average. This high fatality rate is driven by the health zones of **Beni** (75.0%), **Katwa** (66.7%), and **Oicha** (66.7%). The Butembo health zone, despite having more cases (n=25), has a slightly lower case fatality rate (44.0%). The presence of cases with a 100% case fatality rate in sparsely populated areas (Vuhovi, Musienene) is an indicator of the virus's danger in this region and the need for an immediate clinical response.

4. RESPONSE ACTIONS BY PILLAR

4.1 Coordination

National / Ituri:

- Arrival of 3 national ministers including the one in charge of public health, hygiene and prevention, humanitarian affairs and social actions as well as the one of communication to assess the actions of the response in the province of Ituri, one month after the official declaration of the epidemic.
- Holding of the coordination meeting on monitoring the activities carried out in the field by the partners technical and financial aspects across the pillars of the response.
- Held a working session with the new military governor of Ituri province.

4.2 Epidemiological surveillance

Table 3. Management of epidemiological alerts (24h) as of June 18, 2026

Indicator		Ituri	North Kivu	South Kivu	Total
Alerts raised		436	681	22	1139
Alerts investigated		159	681	22	862
Investigation rate		36.5%	100.0%	100.0%	75.7%
Alerts confirmed	Alive	119	55	10	184
	Deceased	30	24	0	54
Total suspected cases for today		149	79	10	238

By the end of June 18, 2026, **1,139 alerts** had been received in the three affected provinces, **862 (75.7%)** of which were investigated, and **238 suspected cases (54 deaths)** were identified. A low rate of investigated alerts was observed in Ituri province.

Table 4. Contact tracing of confirmed cases as of June 18, 2026

Province	Contacts under follow-up (n)	Contacts viewed (n)	Follow-up rate (%)
Ituri	5,155	3,961	76.8%
North Kivu	1821	1047	57.5%
South Kivu	118	118	100.0%
Total	7094	5,126	72.3%

- Continued in-depth investigations and exhaustive listing and monitoring of contacts around confirmed cases in the ZS (narratives of confirmed cases, number of contacts listed and monitored per case).
- 338 new contacts have been listed in 5 health zones in Ituri province. These contacts are distributed in the ZS of Bunia (117), Rwampara (102), Mongbwalu (93), Lita (15) and Bambu (11).

Update on checkpoint and point of entry (PoC/PoE) activities

- **PoC Mudzibala/Bunia** : 2 alerts were notified, including 1 live alert from the Kilo health zone which was investigated, validated and referred to the Bunia medical center and 1 lifeless body from CH Muzimaria to Central soleniama, was released after several hours of waiting without being swabé.
- **PoC Route Watsa/ARU** : 1 lifeless body intercepted, notified after verification of documents, it was released, for the cemetery.

Table 5. Monitoring of indicators at Points of Entry and Points of Control (PoE/PoC) in Ituri province, as of June 18, 2026

Indicators	Results
Proportion of activated PoCs	80.0% (44/55)
Number of travelers who passed	60,901
through, % of travelers	95.0%
screened, % of travelers who washed their	94.0%
hands, % of travelers made aware of the risks	97.0%
Number of alerts notified	3
Number of validated alerts	3
% of results available in 24 hours	0%
Number of alerts transferred to the CT/CTE or for EDS	1
Number of contacts intercepted today	0

4.3. Laboratory

- **Ituri**: 101 samples collected and analyzed (positivity 34.6%; n=35).
- **North Kivu** : 79 new samples collected and analyzed (positivity 1.3%; n=1).
- **South Kivu** : 10 new samples collected and are being analyzed.

4.4. Infection Prevention and Control (IPC)

Ituri: A briefing of 40 service providers was organised in the Mambasa and Mongbwalu, Bunia and Komanda health zones on standard precautions (correct hand washing, wearing and removing PPE, biocleaning); decontamination of the Komanda and Bunia health zones; provision of 4 Wash kits in the Bunia and Mongbwalu health zones.

- **North Kivu:** Assessment of the degree of exposure of healthcare providers at the CBCA Hospital in the Katwa Health Zone: 1 at high risk; scorecard assessment of 3 health facilities in the Beni Health Zone (CBCA Hospital 56.5%, Heshima Letu Hospital 60%, Bora Uzima Dispensary 22%). Gaps: Non-functional triage, insufficient handwashing facilities, insufficient PPE, untrained providers, poor waste management, and lack of an incinerator; briefing of 222 healthcare providers on infection prevention and control (IPC) topics in the health zones of Beni, Butembo, Goma, Katwa, Karisimbi, and Nyiragongo; 46 health facilities supported in IPC as part of enhanced mentoring following IPC scorecard assessment; decontamination of 6 health facilities, including 3 in Katwa, 1 in Butembo, and 2 in Beni, after the visit of confirmed cases.
- **South Kivu:** Packaging and distribution of IPC and PPE inputs in the Miti-Murhesa Health Zone; support to the EIR (donning and doffing of PPE) for 10 samples; decontamination of 21 households including 8 Kalangane Health Zones (Katana Health Zone) and 13 Kabushwa Health Zones (Miti-Murhesa Health Zone); installation of 4 Checkpoints including 3 in the Lwiro Health Zone and 1 in the Kabushwa Health Zone with the support of PNHF/IOM; completion of the distribution of IPC kits in schools (KABINDI Primary School, MURHANGA Primary School and CIBATI/COMBO Primary School); support for the decontamination of the ambulance after the transfer of patients; Scorecard carried out at the Bugarula Hospital Center Idjwi Health Zone with a score of 43.5%.

4.5 Holistic care

- **Two (2) patients** with Ebola virus disease have been declared cured after obtaining negative control tests.
- Continued nutritional monitoring of patients in the CTEs: 569 hot meals were served, including 145 among confirmed cases.

Table 6. Patient movement within healthcare facilities (CTE/CT/CI) as of June 18, 2026

Indicator	Ituri	North Kivu	South Kivu	Together
Patients in bed (Day -1)	335	48	15	398
Total admissions (24h)	53	8	9	70
Exits (24h)				
Deceased (Suspects/Confessions)	16	0	0	16
No case	19	0	13	32
Healed	2	0	0	2
Escapees (Suspects/Confessions)	2	0	0	2
Transferred to the HGR	0	0	0	0
Total outflows	39	0	13	52
Patients in isolation (End of Day),	349	56	11	416
including confirmed cases	139	22	1	162
(NC+AC) and suspected cases	210	34	10	254
Overall occupancy rate	103.3%	78.9%	34.4%	94.3%

Mental health activities and psychosocial support

Table 7. Key indicators of mental health and psychosocial support activities as of June 18, 2026

KEY INDICATORS	ITURI	SOUTH KIVU	NORTH KIVU
New confirmed cases admitted to the CTE who benefited from SMSPS interventions	3 (100%)	5 (45.5%)	0
Confirmed cases currently being monitored that have received interventions SMSPS	49 (57.6%)	11 (36.7%)	0
New suspected cases that benefited from SMSPS interventions	3 (50%)	4 (40%)	1 (100%)
Suspected cases under follow-up that have received interventions SMSPS	53 (80.3%)	24 (36.9%)	0
Laboratory results have been announced.	10	0	3
Including positive ones	3	0	0
Number of children separated in daycare who benefited from interventions SMSPS	1	0	0
Number of children separated in the community who benefited from SMSPS interventions	0	6	0
Number of accompanying persons at the CTE who benefited from interventions SMSPS	71	52	0
Number of PPLs who benefited from SMSPS interventions	35	28	25
Number of household and community members who benefited from SMSPS interventions	37	12	0
Number of EDS supported	2	0	0

4.6 Continuity of care

• Nothing to report

4.7 Risk communication and community engagement

North Kivu: 7,732 households visited out of 15,990 (48%) / 14,949 people sensitized (9,010 F / 5,939 H): Faced with this insufficient coverage, a catch-up plan aims for 75% of households by June 30 with 10 additional mobile teams; 342 religious leaders and 58 APA involved in 18 radio broadcasts plus 8 rebroadcasts reaching 340,000 listeners: joint UNICEF/WHO/ZS Katwa visit plus 61 local authorities mobilized in the Karisimbi ZS; 81 key actors trained (35 RECO, 12 teachers, 10 traditional healers, 8 pastors, 10 pharmacists, 6 managers) with an increase in knowledge of more than 32% (target of more than 50%).

South Kivu: Advocacy with 15 teachers to encourage them to strengthen awareness in classrooms and in the community; briefing of 64 community leaders, community relays and PRECO on CREC interventions around cases and communication techniques; 748 households visited, 5,483 people sensitized in households on prevention measures, the importance of household decontamination and EDS; 2 radio broadcasts on prevention in the Miti-Murhesa health zone; 64 leaders engaged in prevention.

Ituri: Community dialogue with 46 affected individuals, including 8 men and 38 women, on adherence to Ebola prevention measures, care, community alerts, and EDS in the health zones of Mongbwalu and Nyankunde; 1,318 people affected, including 1,288 men and 30 women; home visits to 685 households by Recos with messages on Ebola prevention measures and reporting of alerts (deaths and survivors) affecting 3,186 people (1,339 men and 1,847 women) in the Aru health zones, Bunia, Nyankunde, Rwampara; educational talks organized in 246 households enabled to reach 2,400 people (1,049 men and 1,351 women); briefing of 120 community health workers in the Bambu and Nia-Nia health zones on community-based surveillance; training of 23 service providers on community-based surveillance

community in the Bunia health zone; dissemination of spots and messages in 18 different media on preventive measures for Ebola; continuation of media broadcasts on preventive measures for Ebola.

Support to other pillars : Surveillance : listing of 43 contact cases around a suspected Ebola death (AS CECA 20, ZS Mongbwalu); support for investigations in the AS Shari, Ngona, Tchunga, Hoho, PF Rwampara, Dele, Bogoro health zones) in the Rwampara health zone. **IPC :** Decontamination of 4 households and 4 facilities.

4.8. Logistics

- Delivery of IPC inputs to the health zones of Boga, Nizi, Niania, Nyankunde and to the displaced persons sites of Kigonze and ISP, of medicines to the Bambu health zone, of dignity kits and IPC inputs to the CTE CME Bunia, CTE HGR Nyankunde, CSR Shari, of small equipment and IPC inputs to the Provincial Lab/INRB.

4.9. Security

- The security situation remains calm but unpredictable in Bunia and surrounding areas.

4.10. PSEA (Prevention of Sexual Exploitation and Abuse)

Ituri : Briefing of 208 actors including 99 women on the PSEA and the code of good conduct; 42 including 30 men and 12 women Red Cross volunteers in the Bunia health zone were briefed on the code of good conduct.

The heads of the neighborhoods and avenues in the communes of Shari and Mbunya, within the Bunia and Rwampara Health Zones, signed the codes. A total of 46 participants (22 women, 24 men) participated; awareness-raising was conducted with 80 community health workers (46 men and 34 women) in the Beni Health Zone; a radio program broadcast in Swahili on RTNC Bunia, co-hosted with the Red Cross, was broadcast on the theme: "Why PEA in the Ebola Virus Disease Response"; awareness-raising was conducted with 63 people (13 women and 50 men) on PEAHS in the Mongbwalu Health Zone; and 13 healthcare providers were briefed in the Komanda and Fataki Health Zones during training on SONU.

5. MAIN CHALLENGES, IMPACTS AND REQUIRED ACTIONS

Challenge	Impact	Actions required
No. 1 Reluctance to undergo post-mortem sampling (EDS/swabs) and community resistance	Confirmation delays, underestimation of cases, disruption of key activities	Intensify CREC, involve religious and community leaders, and strengthen team security.
2 Insufficient capacity in CTE and saturation in Ituri (103.3%)	Admission delays, increased risk of nosocomial transmission	Accelerate the construction and expansion of CTE, deploy additional temporary structures
3 Poor performance in contact tracing (72.3% vs target 95%)	Uninterrupted transmission chains	Train, deploy and motivate community liaisons (RECOs), enhanced supervision
4. Insufficient surveillance (36.5% of alerts investigated in Ituri)	Undetected transmission, silent spread of the epidemic	Strengthen community surveillance, improve the reporting and rapid investigation of alerts
5 confirmed cases not localized or unclassified (~17 cases)	Major gaps in the reconstruction of transmission chains	Conduct a data reconciliation process to prune and eliminate potential duplicates, and reclassify or clarify the health zone and status (deceased, recovered, active, etc.). Active verification in the field.
6 Insufficient supply of MEG and medical supplies	Weakening of overall care	Advocacy and urgent supply of essential medicines

7	Insufficient PCI inputs (PPE, chlorine) and limited training	High risk of nosocomial infections (75 cases) HCW, lethality 22.6%	Urgent supply, strengthening of IPC training
8	Stockpile of unanalyzed samples (backlog) at the laboratory in North Kivu and delayed diagnosis	Delayed confirmation, high lethality (58.4%)	Deploy laboratory catch-up plan, strengthen diagnostic capabilities
9	Insufficient number of ambulances and isolation facilities (gap of 20 centers)	Transfer delays and isolation increase the risk of transmission	Urgent deployment of additional logistical resources and infrastructure
10	Insufficient and weak coordination use of COUSP frames	Operational inefficiency and delayed decisions	Strengthen coordination mechanisms at all levels, operationalize the COUSP
11.	Insufficient resources financial (gap of 20 million USD)	Limitation of the implementation of all pillars of the response	Urgent mobilization of resources, advocacy with partners
12	Insecurity and limited access (CODECO, ADF)	Restrictions on operations, limited access to high-risk areas	Strengthening security coordination and humanitarian access
13.	Low increase in alerts (particularly in South Kivu)	Delay in case detection	Strengthening community surveillance and alert systems
14.	Continuity of Essential Services (CEHS) Compromised	Increase in indirect mortality (not Ebola)	Effective implementation of free healthcare, strengthening of CEHS
15.	High population mobility	High risk of rapid geographic expansion	Strengthening surveillance at PoE/PoC sites and cross-border coordination

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